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/ **Practice Exam - 1**

Correct Answer

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Practice Exam - 1

Question 86 of 117



A 55-year-old patient is referred to your clinic for evaluation of left ventricular hypertrophy. The patient's past medical history includes hypertension, diabetes mellitus, and obesity. Symptoms include progressive exertional dyspnea and chest pressure over the prior 3 years especially with strenuous activities. Nuclear stress test and coronary angiography were consistent with the absence of obstructive coronary artery disease.

Vital signs in clinic are: HR 90 bpm, BP 102/70 mm Hg, BMI 31 kg/m². On examination, jugular venous pressure is normal and there is no peripheral edema.

Medications include: aspirin 81 mg/day, pravastatin 40 mg/day, metformin 1000 mg/day, lisinopril 10 mg/day. Echocardiogram shows left ventricular hypertrophy with interventricular septum measuring 21 mm and LV posterior wall 18 mm. The patient is noted to have a resting LVOT gradient of 8 mm Hg that increases to 82 mm Hg with the Valsalva maneuver, along with concomitant systolic anterior motion of the mitral valve.

What is the best first step in management?

A. Initiate verapamil ER 180 mg/day

x **B.** Refer for septal myectomy

C. Refer for alcohol septal ablation

✓ **D.** Discontinue lisinopril 10 mg/day

E. Initiate disopyramide 600 mg/day

Commentary

References

Testing point: The Board-certified AHFTC specialist should be able to identify which is the first therapy action to be undertaken in a patient with hypertrophic cardiomyopathy.

Answer: D. Discontinue lisinopril 10 mg/day.

This patient is referred for what was thought to be left ventricular hypertrophy and has been managed as hypertension in the setting of other cardiovascular risk factors. However, the patient does not have very high afterload, and LVH is out of proportion to blood pressure. Instead, hypertrophic cardiomyopathy is present, as evidenced by the severe wall thickening, somewhat asymmetric pattern, LVOT gradient that is especially pronounced with Valsalva, and systolic anterior motion of the mitral valve. Given the tendency to exhibit inducible obstruction of the LVOT, afterload reducing agents should first be withdrawn, and the patient should be counseled about the importance of hydration. These efforts help reduce the tendency to precipitate LVOT obstruction and exertional symptoms.

Answer A: After removal of offending issues, the next step in the management of obstructive HCM would be to initiate verapamil ER.

Answer E: Disopyramide should only be considered if beta blockers or calcium channel blockers are first tried and maximized.

Answers B and C: Even though the provoked gradient is reasonably high, interventional and surgical therapies are considered after failure of medical therapies.

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